

BUSINESS CASE

Referral Process Optimisation

The Sydney Private Hospital, Ashfield NSW 2131

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1. Executive Summary

The Sydney Private Hospital (TSPH) is an 88-bed private surgical and rehabilitation centre in Ashfield, NSW, providing healthcare services to the Inner West for over 90 years. TSPH currently receives approximately 100 referrals per month across phone, email, fax, online portal, and walk-in GP channels.

The hospital's current referral response time exceeds 2 hours — well above the 30-minute expectation of referring hospitals and the 15-minute target set by the Director. This delay is causing TSPH to lose referrals to competitor hospitals, directly impacting revenue and patient throughput. Based on conservative estimates, the hospital processes approximately 100 referrals per month.

This business case proposes a Referral Process Optimisation solution that introduces a dedicated referral line, a central referral queue, a pre-established eligibility screening process, and digital bed confirmation and approval workflows. The proposed solution targets a response time of under 15 minutes — an 87% improvement on the current state.

Metric	Current	Target	Improvement
Avg response time	2+ hours	Under 15 min	87%
Track 1 (Rehab 70%)	2+ hours	Under 5 min	96%
Track 2 (Surgical 30%)	2+ hours	Under 15 min	87%
Weighted avg response	2+ hours	~7 min	94%
Referrals lost to delays	Est. 15–25%/month	Near zero	Revenue recovered

2. KPI Framework

2.1 Referral Performance KPIs

To measure the success of the referral optimisation initiative, a set of key performance indicators (KPIs) has been defined to track operational efficiency, referral conversion, and overall service performance.

KPI	Definition	Current	Target	Owner
Response Time	Time from referral received to first response	2+ hours	<15 min	Referral Coordinator
Conversion Time	% referrals converted to admission	~ 75-85%	>95%	NOM
Track 1 Speed	Rehab approvals	2+ hours	<5 min	Coordinator
Lost Referrals	% redirected elsewhere	15-25%	<5%	Director
Bed Utilisation	Occupancy rate	Unknown	+10-15%	Bed Manager

3. Problem Statement

3.1 Hospital Background

The Sydney Private Hospital is located at 63 Victoria Street, Ashfield NSW 2131. It operates 88 beds, 8 operating theatres (3 dedicated to eye surgery), 4 fully equipped gyms, a hydrotherapy pool, and a multidisciplinary allied health team including physiotherapy, occupational therapy, dietetics, speech pathology, and social work.

The hospital serves a predominantly rehabilitation patient cohort (estimated 70% of admissions) with the remainder being surgical cases. It receives referrals from hospitals, GPs, and specialists across the Inner West and greater Sydney.

3.2 The Referral Problem

Referring hospitals — including their Nursing on Manager (NOM), bed managers, and admitting doctors — expect a referral response within 15–30 minutes. When TSPH fails to respond within that window, referring hospitals redirect their patients to competitor facilities. This results in lost admissions, lost revenue, and reduced bed occupancy.

The root cause is a fragmented, manual referral intake process with no central coordination. Referral calls enter through the main hospital switchboard with no dedicated routing, resulting in misdirection, extended hold times, and repeated callbacks. Once a call reaches the NOM, further delays occur chasing bed availability and doctor approval by phone — all manual, all untracked.

3.3 Current State Pain Points

ID	Pain Point	Delay	Impact
PP-01	No dedicated referral intake channel	15–25 min	Calls misdirected or put on hold before process starts
PP-02	Reception has no referral protocol	10–20 min	Inconsistent routing — sometimes NOM, sometimes on hold
PP-03	Verbal and paper information capture	10–15 min	Information repeated, errors, no audit trail
PP-04	Manual ward-by-ward bed check	30–40 min	Longest single delay — no live bed visibility
PP-05	Doctor hard to reach by phone	30–45 min	Single biggest bottleneck — no digital approval pathway
PP-06	No triage — all referrals treated equally	Entire flow	70% routine rehab referrals go through full approval unnecessarily
PP-07	Manual HIS entry after each referral	5–10 min	Error prone, no real-time data for reporting

4. Proposed Solution

4.1 Solution Overview

To address the delays, inefficiencies, and revenue loss identified in the current referral process, this proposal introduces a centralised, digitally enabled referral management model designed to streamline intake, accelerate decision-making, and improve visibility across all stakeholders.

The future-state solution replaces fragmented, manual workflows with a single point of coordination, supported by structured triage logic, real-time data visibility, and automated communication.

At its core, the solution is built on three principles:

- Speed — reduce referral response time from over 2 hours to under 15 minutes
- Simplicity — eliminate unnecessary steps for routine cases (70% of referrals)
- Visibility — provide real-time tracking and accountability across the process

This is achieved through a combination of process redesign, role clarity, and targeted digital enablement, rather than reliance on technology alone

4.2 Operating Model

The success of the proposed solution depends on a clearly defined operating model, with centralised ownership, streamlined responsibilities, and aligned service-level expectations. The future-state introduces a Referral Coordinator function as the single-entry point for all referrals, ensuring consistency, accountability, and speed from the moment a referral is received.

Roles and Responsibilities

Role	Responsibility	Key SLA
Referral Coordinator	Owns all referral intake, triage, and routing	1-2 min
Nursing Unit Manager	Clinical oversight and escalation point	Immediate
Bed Manager	Confirms real-time bed availability via live dashboard	1 min
On-Call Doctor	Provides approval for surgical/complex cases (Track 2)	5-10mins
System	Enables auto-logging, tracking, notifications and data capture.	Instant

This model removes dependency on ad hoc communication and introduces a structured, accountable workflow ensuring that each referral progresses efficiently without delays or ambiguity in ownership.

4.2 The 3-Track Triage System

To eliminate unnecessary delays and align effort with clinical complexity, referrals are categorised into a three-track triage model based on predefined eligibility and exclusion criteria.

This approach ensures that the majority of referrals are processed rapidly, while maintaining appropriate clinical governance for complex cases.

Track	Type	Criteria	Volume	Target
Track 1	Rehabilitation — auto approved	Meets established eligibility criteria and can be confirmed without doctor's approval	70% (~70/month)	Under 5 min
Track 2	Surgical — digital approval	Surgical / complex case. NUM sends digital request to on-call doctor for approval via phone/app.	30% (~30/month)	Under 15 min
Track 3	Excluded — immediate decline	Meets published exclusion criteria (medically unstable, palliative, severe cognitive impairment). Declined at Coordinator level.	Est. 5% of referrals	Under 2 min

Key Benefits of the Triage Model

- Reduces bottlenecks by removing unnecessary doctor involvement for routine cases
- Accelerates decision-making through predefined clinical rules
- Improves consistency in referral handling and outcomes
- Prioritises clinical effort where it adds the most value

This triage model is central to achieving the targeted reduction in response times and ensuring scalability as referral volumes increase.

4.3 Key Solution Components

The future-state operating model is enabled by a set of integrated process and technology components designed to support speed, accuracy, and visibility.

1. Dedicated Referral Intake Channels

- Introduction of a dedicated referral phone line with IVR routing
- Standardised intake across email, fax, online portal, and walk-in referrals
- Ensures all referrals are directed into a single coordinated workflow

2. Central Referral Queue

- A unified system where all referrals are captured, timestamped, and prioritised
- Provides full visibility of referral status across stakeholders
- Eliminates lost, duplicated, or delayed referrals

3. Structured Triage and Eligibility Screening

- Referral Coordinator applies predefined clinical criteria at intake

- Immediate categorisation into Track 1, 2, or 3
 - Reduces unnecessary escalation and processing time
4. Digital Doctor Approval Workflow
 - Structured referral summary sent to the on-call doctor via mobile or web interface
 - Enables rapid approval or decline without phone-based delays
 - Reduces approval time from up to 45 minutes to under 10 minutes
 5. Live Bed Management Dashboard
 - Real-time visibility of bed availability across all wards
 - Enables instant confirmation without manual checks
 - Removes the longest delay in the current process
 6. Automated HIS Integration
 - Referral data captured once at intake and auto-populated into the Hospital Information System (HIS)
 - Eliminates duplicate data entry and reduces errors
 - Provides a complete digital audit trail
 7. Automated Referral Confirmation
 - Instant confirmation sent to referring hospitals upon decision
 - Includes bed allocation, admission details, and estimated timelines
 - Improves referrer experience and strengthens referral relationships

4.4 Expected Outcomes

The implementation of this solution is expected to deliver significant operational and financial improvements:

- Reduction in average referral response time from over 2 hours to under 15 minutes
- Near elimination of lost referrals due to delayed responses
- Improved bed utilisation and patient throughput
- Enhanced stakeholder experience for referring hospitals and clinicians
- Full process visibility and auditability through digital tracking

Importantly, this is not solely a technology initiative. It represents a fundamental shift in how referrals are managed, moving from a reactive, manual process to a proactive, structured, and data-driven operating model.

5. Process Diagrams (As-Is & To-Be)

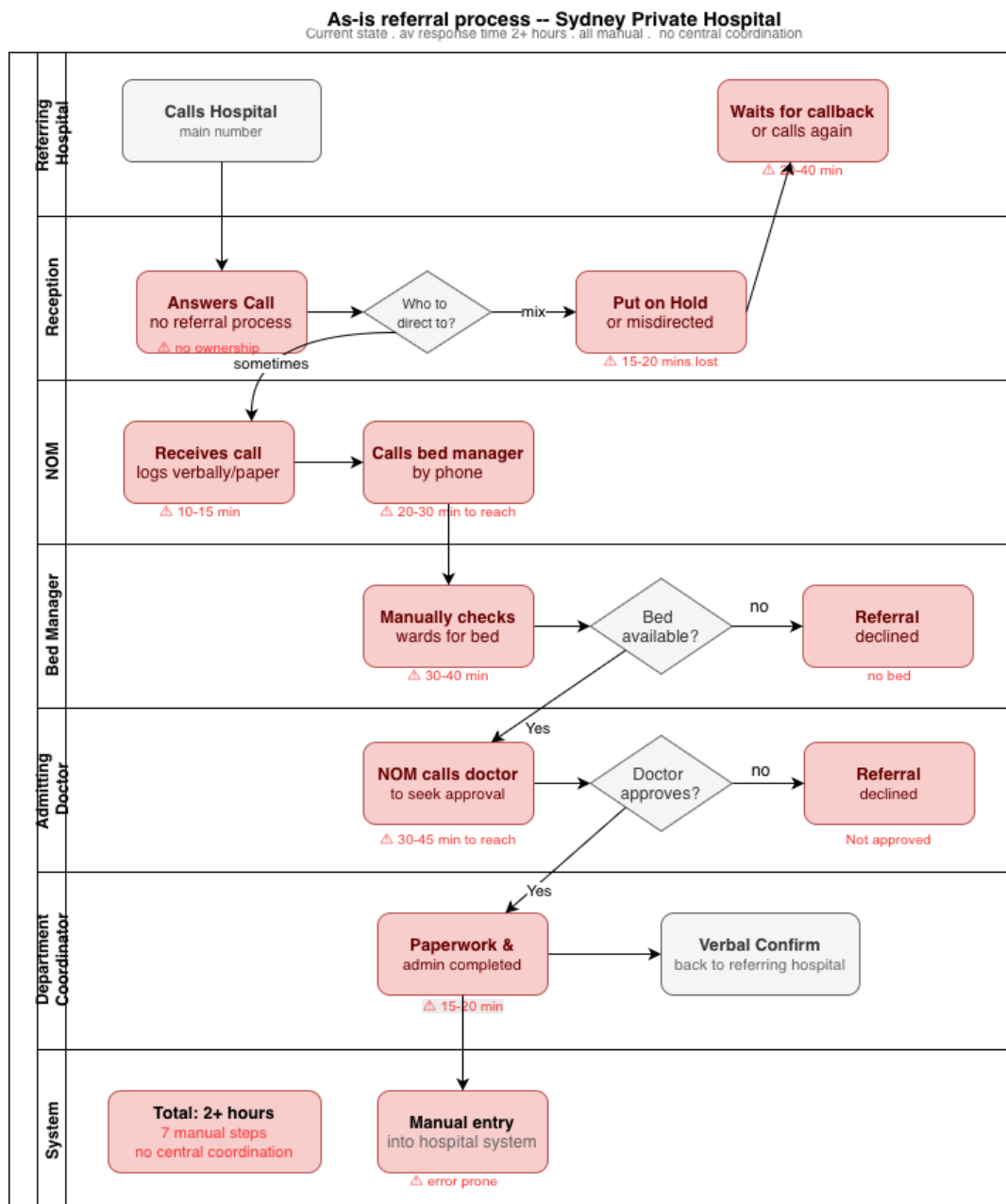
To provide a clear understanding of the current inefficiencies and the proposed improvements, the referral process has been mapped in both its current state (As-Is) and future state (To-Be).

The As-Is process highlights key delays, manual dependencies, and lack of coordination across referral intake, triage, and decision-making.

The To-Be process illustrates a streamlined, centralised workflow supported by defined roles, structured triage logic, and digital enablement. It incorporates service-level expectations, clear ownership, and system-supported steps to ensure referrals are processed efficiently and consistently. Together, these diagrams demonstrate the transformation from a fragmented, reactive process to a structured, time-bound, and scalable operating model.

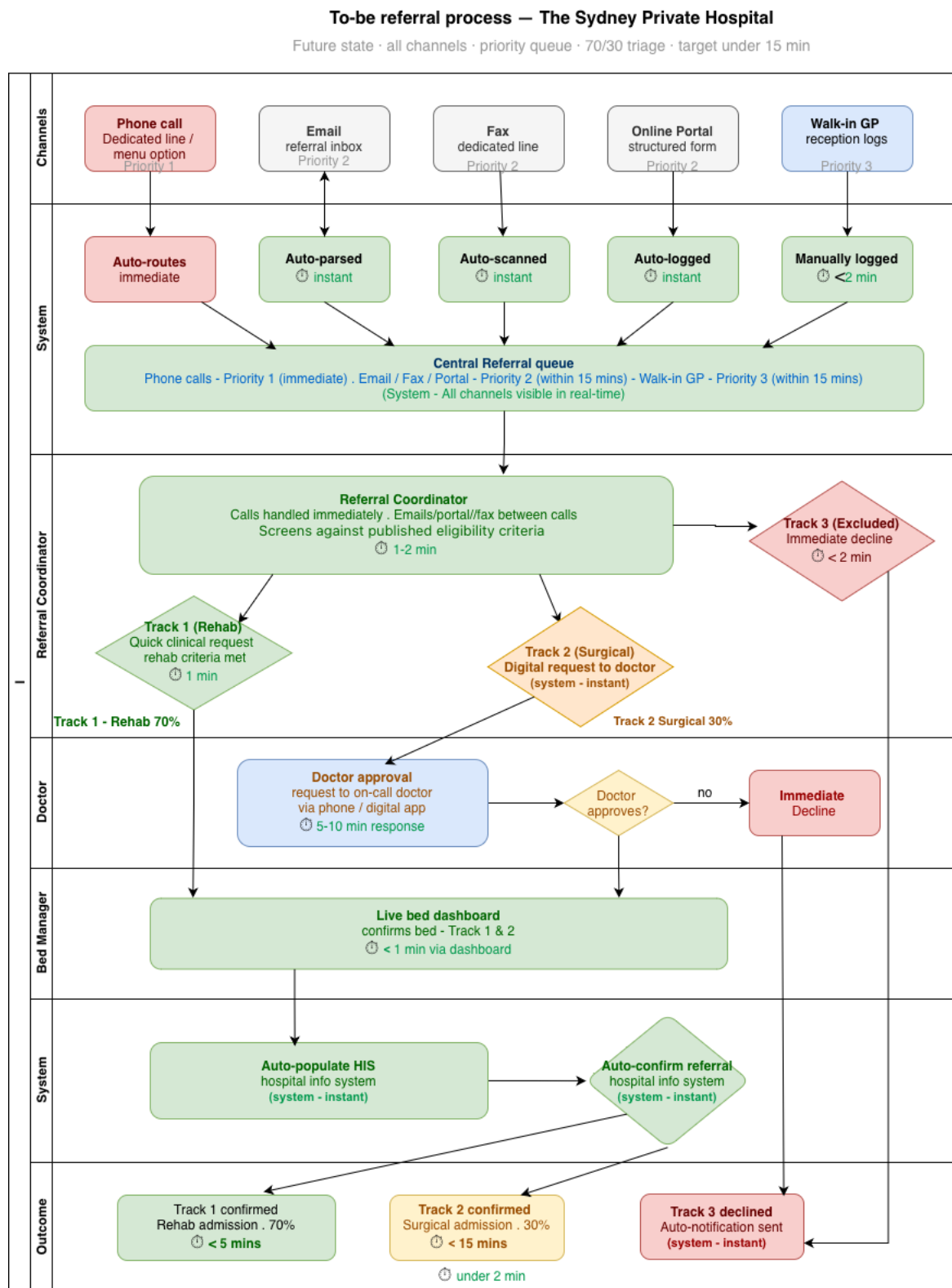
5.1 As-Is Process

Current State Referral Workflow



5.2 To-Be Process

Future State Referral Workflow (Target: <15 min response time)



6. Cost-Benefit Analysis

6.1 Assumptions

- Conservative referral volume used: 100 referrals per month
- Estimated 15% of referrals currently lost due to slow response time (15 referrals/month)
- Average revenue per admission (rehab): AUD \$8,000–\$12,000 (conservative estimate for private hospital)
- Average revenue per admission (surgical): AUD \$15,000–\$25,000
- Weighted average revenue per recovered referral: AUD \$10,000
- Staff time costed at AUD \$45/hour (coordinator/NOM blended rate)

6.2 Cost of the Problem (Monthly)

Cost Item	Monthly Estimate	Basis
Lost referrals (15% of 100)	AUD \$150,000	15 referrals x \$10,000 avg revenue
Staff time on manual intake (NOM + reception)	AUD \$3,375	100 referrals x 45 min avg x \$45/hr
Rework and error correction (HIS, paperwork)	AUD \$675	Est. 15% of referrals require rework x 1hr x \$45
TOTAL MONTHLY COST OF PROBLEM	AUD \$154,050	Conservative estimate — 100 referrals/month

6.3 Implementation Cost (One-off)

Investment Item	Estimated Cost	Notes
Dedicated referral line + IVR setup	AUD \$5,000–\$10,000	Telco setup, one-off configuration
Central queue software / referral management system	AUD \$20,000–\$40,000	Off-the-shelf or existing system configuration
Live bed dashboard (HIS integration)	AUD \$15,000–\$30,000	IT development, depends on existing HIS
Digital doctor approval workflow	AUD \$10,000–\$20,000	App or SMS-based approval integration
Staff training and change management	AUD \$5,000–\$10,000	Coordinator role, NOM training, protocols
TOTAL IMPLEMENTATION COST	AUD \$55,000–\$110,000	One-off investment

6.4 Return on Investment

Metric	Conservative	Optimistic
Referrals recovered per month	10 (10% of 100)	20 (20% of 100)
Monthly revenue recovered	AUD \$100,000	AUD \$200,000
Annual revenue recovered	AUD \$1,200,000	AUD \$2,400,000
Implementation cost (one-off)	AUD \$110,000	AUD \$55,000
Payback period	~5 weeks	~2 weeks
ROI (Year 1)	991%	4,264%

At even the most conservative estimate, the implementation pays for itself in under 5 weeks of recovered referrals. The ongoing annual benefit far exceeds the one-off investment, making this one of the highest-return operational improvements available to TSPH.

7. Implementation Roadmap

Phase	Timeline	Activities	Expected Outcome
Phase 1 Operational Reset / Quick win	Weeks 1–4	Establish referral protocol for reception. Designate a Referral Coordinator. Create triage criteria document based on published eligibility guide. Train NUM on 3-track logic.	Immediate improvement in call routing. Referral response time drops to 60–90 min with no technology investment.
Phase 2 Workflow Digitisation Core system	Weeks 5–12	Install dedicated referral phone line and IVR menu. Deploy central referral queue system. Implement digital doctor approval workflow. Launch online referral portal.	Response time drops to under 15 minutes. All channels centralised. Track 1 and Track 2 fully operational.
Phase 3 System integration	Weeks 13 - 20	Integrate live bed dashboard with HIS. Automate HIS population from referral intake. Configure automated confirmation back to referring hospitals. Build referral reporting dashboard.	Zero manual data entry. Full audit trail. Full automated process
Phase 4 Optimisation and Scale	Weeks 21 - 24	KPI dashboard (Power BI) Performance Monitoring Continuous improvement loop Scale to higher referral volumes	Director-level KPI reporting on referral volume, response time, and conversion rate.

Phase 4 ensures the solution evolves from implementation to a continuously optimised operational capability.

8. Delivery Plan

8.1 Agile Delivery Plan

Phase	Timeline	Activities / Focus	Expected Outcome
Sprint 0	2 weeks	Stakeholder alignment + process design	Agreed workflow + buy in
Sprint 1	2 weeks	Coordinator setup + triage pilot	Manual process live
Sprint 2	2 weeks	Digital intake form + central queue	Structured intake
Sprint 3	2 weeks	Tracking + visibility dashboard	Transparency
Sprint 4	2 weeks	Doctor approval workflow	Faster decisions
Sprint 5	2 weeks	Stabilisation + user feedback	System refined
Sprint 6	2 weeks	Bed management dashboard	Real-time allocation
Sprint 7	2 weeks	HIS integration (phase 1)	Data sync begins
Sprint 8	2 weeks	HIS integration (phase 2)	Full automation
Sprint 9	2 weeks	Notifications + system hardening	Reliable operations
Sprint 10	2 weeks	KPI dashboard (Power BI)	Performance Visibility
Sprint 11	2 weeks	Process optimisation + scale readiness	Sustainable Model

The delivery plan spans 12 two-week sprints across a 24-week period, fully aligned to the implementation roadmap and covering design, build, integration, and optimisation phases.

9. Risks and Assumptions

9.1 Assumptions

- Referral volume of 70-100 per month is sustained or grows following process improvement
- The hospital's existing HIS supports API integration for bed dashboard and auto-population
- An on-call doctor roster system is available or can be established for digital approval routing
- Budget approval for Phase 2 technology investment is granted prior to Week 5
- Referring hospitals will increase referral volume once response times improve — based on the Director's assessment that slow response is the primary barrier

9.2 Risks and Mitigations

ID	Risk	Strategic Impact	Mitigation
R-01	Rapid increase in referrals post-improvement	Operational Overload	Phase rollout + capacity planning
R-02	Single point dependency (Coordinator)	Process bottleneck risk	Cross-training + backup roster
R-03	Clinical resistance to new workflow	Adoption failure	Early engagement + pilot program
R-04	Technology integration delays	Timeline Slippage	MVP approach before full integration
R-05	Data quality issues at intake	Poor decision-making	Outside scope of this project. Flagged for separate review. Does not affect Track 1 (70% of volume) at all.
R-06	Over-reliance on digital approval	Doctor fatigue	Escalation rules + prioritisation logic

The primary risk is not technology, but adoption and operational scalability.

10. Recommendation

It is recommended that The Sydney Private Hospital proceeds with the three-phase Referral Process Optimisation program as described in this business case.

Phase 1 requires no technology investment and can begin immediately. The appointment of a designated Referral Coordinator and the implementation of the 3-track triage protocol will deliver measurable improvement within the first month.

The financial case is compelling. At 100 referrals per month, recovering just 10% of currently lost referrals generates over AUD \$1.2 million in annual revenue against a one-off implementation cost of under AUD \$110,000. The payback period is approximately 5 weeks.

The solution is also strategically important. The Inner West of Sydney has a growing ageing population with significant demand for rehabilitation services. The hospital's existing capability — 88 beds, 4 gyms, hydrotherapy, and 5 rehabilitation consultants — positions it well to capture this demand, provided the referral intake process no longer acts as a barrier.

Recommended Action	Timeline
Approve Phase 1 — immediate process changes	Within 2 weeks of this presentation
Approve Phase 2 budget (AUD \$55,000–\$110,000)	Within 4 weeks
Appoint Referral Coordinator	Within 2 weeks
Engage IT team for HIS integration scoping	Phase 2 kickoff

This is not an IT project — it is a revenue recovery and patient access transformation initiative.

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